

STRATEGIC PRESENTATION DECK • OCTOBER 2026

ElderTech Operating System

An Autonomous Hybrid Platform for Solitary Seniors, Nocturnal Circadian Crises, and Global NRI Diaspora Care.

ORIGIN & INCUBATION

SLIET IIC / Tricity Pilot

TARGET GEOGRAPHY

**Chandigarh "Silent Kothis" &
Diaspora**

CORE ARCHITECTURE

4-Pillar Hybrid Autonomous OS

STATUS

Verified v2.4 Master Specification

What We Are Doing Today: The GoldenCares Baseline

Early proof-of-concept operating in the Chandigarh–Mohali–Panchkula corridor

Current Footprint

Student Companionship

On-demand pairing of university students (Panjab Univ, PEC, Chitkara) with solitary elders for daytime conversations, smartphone tech tutorials, and park walks.

Hourly Pay-Per-Use

Billed at ₹499 per hour on-demand. Students receive a per-session stipend; local auto/fuel travel across Tricity sectors is reimbursed separately.

Tricity Footprint

Active pilot testing with early test households in Mohali Phase 7 and Chandigarh Sectors 8/9/10/11, establishing validated initial consumer demand.

The Demographic Reality: Tricity's "Silent Kothi" Crisis

Demographic Metric	Observed Benchmark	Socio-Economic & Clinical Ground Reality
Aging Index	12.6% aged 60+ in Punjab	Significantly exceeds the national average (Ministry of Health / LASI Wave 1).
Youth Emigration Drain	150,000+ youth / year	Massive brain drain to Canada, UK, and Australia leaving aging parents isolated.
"Silent Kothi" Phenomenon	Sectors 8–11 (Chd), Phase 7 (Mohali)	Retired generals, high court judges, bureaucrats alone in 1-kanal villas with liquid wealth but absolute emotional solitude.

The 8 Fatal Loopholes in the Current Baseline Model

Why on-demand student matching fails at scale in real-world eldercare

8 Critical Failure Modes

1. The Nocturnal Void

Companions leave at 6 PM. Melatonin collapses 60–80%, causing awakenings at 2–5 AM with severe existential dread and panic.

2. Cash Disintermediation

By visit #3, families exchange numbers with students and pay ₹300/hr directly in cash, bypassing the platform and killing LTV.

3. Pay-Per-Use Friction

₹499/hr forces a purchase decision every session. Frugal elders delay booking: *"Let's save money today"*, killing habitual retention.

4. Student Churn Cycle

Semester exams, internships, and holidays pull students away every 4 months. Seniors feel abandoned and reject new strangers.

5. Infantilization Trap

Positioning care as "babysitting lonely elders" triggers intense ego defense from proud retired officers and judges.

6. Broken Margins

At ₹499/hr, student pay (₹300) + commute costs (₹120) leave <₹80 gross margin, incapable of absorbing ₹3k customer acquisition costs.

7. Zero NRI Telemetry

NRI children paying from Toronto receive casual WhatsApp texts instead of objective clinical vitals and cognitive trends.

8. Emergency Liability

Untrained students lack Basic Life Support (BLS) training during acute falls, TIAs, or cardiac events, exposing platform to liability.

The Solution: 4-Pillar Autonomous Hybrid Operating System

Neither pure human nor pure AI — an intelligent hybrid socio-technical stack

Defensible Architecture

 Pillar 1: Sarthi AI

<500ms

VOICE PIPELINE LATENCY

- 24/7 vernacular voice agent (Hindi/Punjabi)
- Proactive check-ins during 2–5 AM insomnia
- Passive vocal biomarker surveillance (tremors, word recall latency)

 Pillar 2: Dignity RM

1:30

FIXED ELDER RATIO

- Permanent, salaried geriatric care manager
- Dual-facing: elders locally + NRI kids abroad
- Gatekeeper: approves all AI orders
- Bi-weekly in-person clinical audits

 Pillar 3: Youth Escorts

Mentee

RELATIONSHIP FRAMING

- Vetted students (PU, PEC, Chitkara)
- Framed as mentees seeking life wisdom
- Reverses infantilization ego defense
- Hospital queues (PGIMER/Fortis), walks

 Pillar 4: Golden Club

3 km

MICRO-POD RADIUS

- Hyperlocal clusters of 8–12 active elders
- Weekly bridge, book clubs, garden tea
- Fosters mutual horizontal peer support
- Prevents platform disintermediation

Dynamic Workers Pool & The RM Approval Firewall

Zero idle payroll combined with human-in-the-loop safety verification

Human-in-the-Loop

THE REAL-TIME DISPATCH FLOW

1. 🗣️ Senior Speaks to Sarthi AI at Bedside
"Mainu knee pain ho rahi hai, spray bhej do te PGIMER OPD lai companion book karo"
▼
2. 🧠 Sarthi AI Generates Pending Action Item
• Flags: ₹450 Volini spray + 3hr OPD Companion
▼
3. 🛡️ RM Reviews & Authorizes in Console
• Checks clinical validity & elder safety
• Prevents fraud, scams, or dementia mis-orders
▼
4. 🚶 Dynamic Escort Dispatched from Pool
• Health-tagged student matched for PGIMER
• Debits pre-funded Family Digital Wallet

⚡ Dynamic Workers Pool: Just-In-Time Supply

- **Zero Fixed Salary Burn:** Avoids Goodfellows' mistake of paying fixed salaries to idle companions. Students and escorts sit in an on-demand, skills-tagged pool.
- **Specialized Dispatch:** Hospital queues at PGIMER \$ o\$ Health escorts; Tech tutor / Chess \$ o\$ Engineering mentees; Mobility errands \$ o\$ Mobility assistants.

🛡️ The RM Approval Firewall: Fraud & Liability Shield

- **The Threat:** Direct AI purchases expose elders to predatory scams, mistaken orders, and adverse drug reactions.
- **The Gatekeeper:** The RM approves every financial debit and service dispatch, protecting both elder dignity and platform liability.

The Dual-Bucket Family Wallet & Modular Add-Ons

Stopping platform cash leakage while preserving elder financial dignity

Fintech Moat

Bucket A: Health & Core Care Wallet

- **Funded & Monitored by NRI Child:** Maintained via Stripe in USD/CAD/GBP or domestic auto-debit.
- **Itemized Transparency:** Covers RM visits, prescription medicine refills, routine lab blood draws, and clinical escorts.
- **Full Telemetry:** Receipts and medical compliance logs shared directly with children.

Bucket B: Dignity Freedom Float (Private)

- **Autonomous Senior Spending:** Fixed monthly pool (₹5,000/mo) pre-authorized for tea companions, nursery visits, and hobbies.
- **Zero Child Surveillance:** Child has **no veto or line-item visibility**, ending the humiliation of adult children questioning elder spending.
- **Preserves Autonomy:** Seniors feel respected, not policed.

Modular Add-On Marketplace (Recurring & On-Demand)

Add-On Category	Service Scope	Operational Delivery Mechanism
Monthly Chronic Care	Cardio & Diabetes management: home blood draws, teleconsult, pill-box audits	Contracted via Tier-1 B2B APIs (Dr. Lal PathLabs, Max Labs) with verified SLAs.
In-Home Physiotherapy	2x weekly geriatric mobility therapy & fall prevention	Vetted freelance physiotherapists verified and monitored by the RM.
Hospital OPD Navigation	Half-day companion escort at PGIMER Chandigarh or Fortis Mohali	Health-tagged youth escort manages registration, queues, and pharmacy collection.

System Stress-Testing: The 7 Loopholes & Patches

Pre-empting operational bottlenecks, legal liabilities, and cross-border frictions

Institutional Rigor

#	Identified Loophole	Operational Failure Mode	Architectural Patch
1	RM Overload Bottleneck	1:30 ratio = 60 home visits/mo. Commute + 30 NRI WhatsApp chats burns out RMs in 90 days.	GEOGRAPHIC MICRO-ZONING RMs restricted to tight sector zone (Chd Sec 1–15 only). Ratio capped at 1:20 for high-acuity tiers.
2	Order Approval Latency	If RM is driving, urgent AI requests (taxi, pain spray) sit unread for 3 hours, frustrating elders.	TIERED APPROVAL THRESHOLDS Orders <₹500 and emergency SOS auto-dispatch instantly. RM review reserved for >₹1,000.
3	Dynamic Pool "Stranger Danger"	Sending different students every week creates theft paranoia; elders refuse to open doors.	PRIMARY + BACKUP POD (2:1) Assign each senior a dedicated 2-person pod. Dynamic pooling applies only to outdoor errands.
4	Family Wallet Dignity Clash	NRI child inspects every rupee; questions dad's spending on snacks, humiliating the proud senior.	FREEDOM FLOAT PARTITION Split wallet into monitored "Health Core" and private "Freedom Float" (₹5,000/mo) with zero child surveillance.
5	AI Misdiagnosis Liability	Elder slurs or uses dialect ("chhati ch jalan"). AI recommends antacids for acute heart attack.	HARDCODED RED-FLAG FIREWALL Chest pain, numbness, or dyspnea locks commercial ordering and triggers instant hospital/RM SOS.
6	NRI Time-Zone Delta	Tricity 1 PM crisis is 12:30 AM in California. Child is asleep; delayed approvals stall medical care.	EMERGENCY POWER OF ATTORNEY Onboarding contract pre-authorizes \$250–\$500 emergency spend threshold for immediate clinical triage.
7	Add-on Vendor No-Shows	Outsourced phlebotomists arrive 2 hours late. Fasting diabetic elder suffers; platform takes blame.	INSTITUTIONAL B2B SLAS Direct API contracts with certified chains (Max Healthcare, Dr. Lal PathLabs) with strict financial penalty clauses.

Global Precedents: Proven in Parts Across the World

Evidence-based validation from peer-reviewed trials and government initiatives

Peer-Reviewed Data

 USA: Papa Pals

-33%

ER ADMISSIONS

Dynamic student companionship funded via Medicare Advantage plans across 50 US states. Proven model for non-medical social support.

 S. Korea: Naver CareCall

20,000+

ELDERS CALLED

Gov-funded telephony AI check-in across 20+ Korean cities (HyperCLOVA LLM). 90%+ retention; automated welfare alerts (ACM CHI 2023).

 USA: NYSOFA & ElliQ

95%

LONELINESS DROP

NY State Government purchased proactive voice AI companions for 800+ seniors. 30+ daily interactions; 80% proactive AI prompts.

 Netherlands: Buurtzorg

-30%

ER VISITS

Decentralized 3km neighborhood nursing pods (our RM layer). 8% overhead vs. 25% industry average; #1 Dutch employer 5 times.

 Empathetic Video Proof & Clinical Studies

• **South Korea: Naver CLOVA CareCall in Action:** Real Korean seniors talking to the AI telephony agent like family, with local welfare officers intervening in emergencies. (YouTube: NAVER CLOVA Official Showcase).

• **New York State: Meet Lucinda (69, New York) & ElliQ:** Moving documentary where Lucinda describes through tears how proactive voice companion ended her nocturnal panic and gave her back joy. (YouTube: Meet Lucinda, 69, from New York ElliQ).

Summary: The 3-Way Lock-in & Anti-Leakage Moat

Why the hybrid operating system cannot be bypassed by clients or workers

The Moat

1. Clinical & Nocturnal Lock-In

- An informal student or maid leaves at 6:00 PM.
- Only Sarthi AI provides the **2:00 AM – 5:00 AM nocturnal shield**, monitoring sleep disturbances and speech biomarkers.
- The family cannot cancel without losing 24/7 emergency telemetry and nighttime security.

2. The RM Trust & Safety Lock-In

- The student is merely an outdoor escort; the **RM is the clinical and emotional guardian**.
- The RM manages hospital emergency triage, medicine reconciliation, and weekly diaspora video digests.
- Disintermediating the student cash-in-hand forfeits the RM safety umbrella and family wallet.

3. Hyperlocal Pod Social Lock-In

- An elder's active social life is rooted in the **Golden Club 3km Pod** (bridge tournaments, poetry sessions, peer friends).
- A family cannot take the 12-person neighborhood club private.
- Social network effects generate organic RWA word-of-mouth with near-zero CAC.